

Exercise Sheet 10

Cascades and Temporal Networks

5942UE Network Science

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10.A Cascades and Threshold Rules

Exercise 10.A.1: Threshold Cascade by Hand

Graph (edges $A-B$, $B-C$, $A-D$, $C-E$, $D-F$) with thresholds: $A(0.4)$, $B(0.5)$, $C(0.3)$, $D(0.5)$, $E(0.5)$, $F(0.3)$

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Seed at $t = 0$: A , C . Synchronous update.

1. Compute the degree of each non-seed node.
2. Trace which nodes adopt at $t = 1, 2, 3$.
3. Does the cascade reach all nodes?

Exercise 10.A.2: Threshold Heterogeneity

Same graph as 10.A.1. Two experiments:

- **A:** All thresholds raised to 0.6.
- **B:** Only B 's threshold raised to 0.6 (others as in 10.A.1).

For seed A, C :

1. Does the cascade reach all nodes in (A)?
2. Does the cascade reach all nodes in (B)?
3. Why does a single threshold change "block" the cascade in (A)?

Exercise 10.A.3: Cascade Simulation

Simulate threshold cascades on `nx.barabasi_albert_graph(100, 3)`.

1. At what seed size does the cascade reliably sweep most of the network?
2. Raise threshold to 0.5 — how does final adoption change?
3. Compare seeding **hubs** (top- k degree) vs **random**.

Exercise 10.A.4: Bridges Differ for Simple vs. Complex Contagion

Dumbbell: two 5-cliques 1..5 and 6..10 joined by bridge 5–6. Node 1 adopts first.

1. **Simple contagion** ($\beta = 1$): does it spread to clique B?
2. **Complex contagion** ($\theta = 0.4$): does it cross the bridge?
3. What change would let the cascade cross?

10.B Temporal Networks

Exercise 10.B.1: Order-Dependent Reachability

Three nodes A, B, C with two timed edges:

1. Can information travel $A \rightarrow C$? Trace the path.
 2. Can it travel $C \rightarrow A$?
- $A-B$ exists at $t = 1$ only
 - $B-C$ exists at $t = 2$ only
3. Reverse the timing ($A-B$ at $t = 2$, $B-C$ at $t = 1$). Does $A \rightarrow C$ still work?
 4. What does this say about temporal vs. static reachability?

Exercise 10.B.2: Static Aggregation Hides Causal Order

A hospital contact network over 3 days:

Day	Edge
1	Nurse–Patient A
2	Nurse–Patient B
3	Doctor–Nurse

1. Can infection spread from Patient A to Doctor? Trace it.
2. Can it spread from Doctor to Patient A?
3. What might static contact tracing wrongly conclude?

Exercise 10.B.3: Temporal Reachability in Code

Implement BFS over a temporal edge list.

1. From node 0, which nodes are reachable and at what earliest time?
2. From node 3, is node 0 reachable?
3. Add edge $(3, 0, 5)$. How does reachability change?

Exercise 10.B.4: When Timing Kills Spreading

Star with hub H and leaves A, B, C, D . Disease starts at A at $t = 0$.

- **Scenario X:** all edges active at $t = 1$
 - **Scenario Y:** $H-A$ at $t = 1$, $H-B$ at $t = 2$, $H-C$ at $t = 3$, $H-D$ at $t = 4$
1. Which nodes get infected in (X)?
 2. In (Y)?
 3. Now suppose the disease has a **1-step infectious window**. In (Y), does it spread beyond H ?
 4. What does this illustrate about epidemic containment?

Wrap-Up

- threshold cascades depend on dense local reinforcement, not just connectivity
- bridges are good for simple contagion but bad for complex cascades
- temporal networks make reachability asymmetric and order-dependent
- contact timing relative to infectious windows can decisively contain spread